

Informed Consent

Properly Documenting Patient Agreement for Procedures

Regulation: 42 CFR §482.13(b) | §482.24(c)

POLICY OVERVIEW

Informed consent is both a legal requirement and an ethical obligation. Patients have the right to make informed decisions about their care, including the right to refuse treatment. The informed consent process must be documented in the medical record.

KEY REQUIREMENTS

1 Disclosure Requirements

Providers must disclose: the diagnosis, proposed treatment/procedure, material risks and benefits, alternatives to proposed treatment, risks of non-treatment, and the identity of practitioners who will perform the procedure.

2 Capacity Assessment

The consenting individual must have decisional capacity. When capacity is in question, a formal assessment must be documented. For patients lacking capacity, the legally authorized representative must provide consent.

3 Documentation Standards

The signed consent form must be present in the medical record before elective procedures. The form must include: date, time, specific procedure, practitioner name, patient/representative signature, and witness signature.

4 Special Consent Situations

Separate consents are required for: blood transfusions, HIV testing, release of records, research participation, photography, and treatment of minors (with appropriate exceptions for emergencies).

5 Right to Refuse

Competent patients have the absolute right to refuse any treatment. Refusal must be documented, including patient education about risks of refusal and the patient's stated reasons.

COMPLIANCE NOTES

Informed consent must be an ongoing process, not a one-time form signing. The physician is responsible for the consent discussion; nursing staff witness the signature. Documentation must reflect the substance of the discussion.

COMMON SURVEY CITATIONS

Cited under §482.13(b)(2) – Right to make informed decisions, and §482.24(c)(2)(v) – Consent documentation in medical records.